February 23, 2026

Submitted electronically via regulations.gov

The Honorable Robert F. Kennedy, Jr.
Secretary of Health and Human Services
Department of Health and Human Services
200 Independence Avenue, S.W.
Washington, D.C. 20201

RE: Public Health Deans and Scholars' Response to Department of Health and Human Services' Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RIN 0955-AA13)

Dear Secretary Kennedy:

The undersigned 125 public health and health policy deans, chairs, and scholars (in their individual capacities) file this response to the Department of Health and Human Services' (HHS) December 23, 2025, request for information about accelerating the adoption and use of Artificial Intelligence (AI) as part of clinical care.[1] Our response addresses several of HHS's questions about the use of AI in the context of healthcare. Specifically, it addresses patient privacy and data confidentiality issues, AI evaluation methods, where AI clinical tools have performed well and where they have fallen short, the challenges and concerns expressed by patients and caregivers, and published findings about AI clinical care tools. We conclude that regulation and oversight are needed by the Department of Health and Human Services to ensure that the use of AI in clinical care reaches its maximum potential to increase access to healthcare and to avoid the potential harms associated with the use of AI in clinical care.

The individual signatories are distinguished deans, chairs, and scholars at the nation's leading academic institutions and research universities. They are experts in the fields of health law, public health, healthcare policy and research, and national health reform. They include individuals known for their expertise in health information privacy and security law and ethics, AI ethics, and health policy regarding clinical care. The individual signatories join this comment in their individual capacities and not as representatives of their respective institutions. The complete list of individual signatories is included at the end of this letter.

Question #3: For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures. What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

Many AI tools currently shaping clinical care are not regulated as medical devices. Examples include patient messaging assistants, coding/prior authorization support, care gap detection, triage/routing, guideline retrieval, ambient documentation, and notes summarization. In practice, these AI tools still influence clinical decisions, clinical documentation, patient decision-making, patient trust, and access to care. Therefore, creating accountability issues that are not well

---

[1] Request for information: Accelerating the adoption and use of artificial intelligence as part of clinical care, *90 Fed. Reg. 60108–60110* (Dec. 23, 2025). https://www.federalregister.gov/d/2025-23641

covered by existing privacy practices, transparency norms, liability frameworks, or civil rights compliance workflows. Key issues are summarized below.

AI's indefinite storage of health data and periodic data leaks can threaten patient privacy.

Training complex AI models depends on the assembly of large collections of health data about patients, which AI models "learn" from and "remember" for future use.[2] Unlike traditional data storage software, this information cannot simply be deleted from AI models.[3] Instead, it is stored indefinitely, and the models internalize the data to identify patterns and generate inferences.[4,5] There are inherent risks associated with this indefinite storage, including the risk of data leaks. Data leaks could expose sensitive health information, threaten patient privacy, lead to personal consequences such as discrimination and stigmatization, and threaten patient trust in the medical profession and public health.

AI data leaks result from common vulnerabilities in AI models, such as bugs and hacks.[6] For example, in 2023, a ChatGPT bug allowed users to see titles from another user's chat history and unintentionally made payment-related information visible.[7] While this bug was quickly patched, experts warn that all AI software systems are built upon other software components that may include weaknesses that can be exploited.[8] In another example, researchers testing the privacy of open-source ChatGPT models found that they could input certain commands that would confuse the model, causing it to release personally identifiable information, including email addresses, names, phone numbers, and pictures.[9]

The integration of AI in clinical settings creates new opportunities for bad actors to launch ransomware attacks, as they may exploit existing bugs, inject prompts that allow them to gain access to databases, or prey on the human errors of users of clinical AI.[10] One analysis of real incidents of AI cybersecurity attacks found that this problem has huge ramifications for

---

[2] Higa, H., Bedikian, S., & Costa, L. (2025, May 20). *The right to be forgotten is dead: Data lives forever in AI.* Tech Policy Press. https://www.techpolicy.press/the-right-to-be-forgotten-is-dead-data-lives-forever-in-ai/
[3] aiunlearning. (2025). *Why deleting data does not remove it from AI models.* Medium. https://medium.com/@aiunlearning/why-deleting-data-does-not-remove-it-from-ai-models-c08c3c37863a
[4] Kak, A. (2025). *Prepared testimony and statement for the record: "The need to protect Americans' privacy and the AI accelerant" before the U.S. Senate Committee on Commerce, Science, and Transportation.* AI Now Institute. https://www.commerce.senate.gov/services/files/35C1CE13-F03D-4CE9-9559-B0651F156D86
[5] Mitchell, H. (2024, January 18). Is it safe to share personal information with a chatbot? *The Wall Street Journal.* https://www.wsj.com/tech/ai/ai-chatbot-sharing-personal-information-229d41a0
[6] Kak, A. (2025). *Prepared testimony and statement for the record: "The need to protect Americans' privacy and the AI accelerant" before the U.S. Senate Committee on Commerce, Science, and Transportation.* AI Now Institute. https://www.commerce.senate.gov/services/files/35C1CE13-F03D-4CE9-9559-B0651F156D86
[7] OpenAI. (2023, March 23). *March 20 ChatGPT outage: Here's what happened.* https://openai.com/index/march-20-chatgpt-outage/
[8] Kaganovich, M., & Chuvakin, A. (2024, December 5). *Oops! 5 serious gen AI security mistakes to avoid.* Google Cloud Blog. https://cloud.google.com/transform/oops-5-serious-gen-ai-security-mistakes-to-avoid
[9] Nasr, M., Carlini, N., Hayase, J., Jagielski, M., Cooper, A. F., Ippolito, D., Choquette-Choo, C. A., Wallace, E., Tramèr, F., & Lee, K. (2023, November 28). *Extracting training data from ChatGPT.* https://not-just-memorization.github.io/extracting-training-data-from-chatgpt.html
[10] Sallam, M., Al-Mahzoum, K., & Sallam, M. (2024). Generative artificial intelligence and cybersecurity risks: Implications for healthcare security based on real-life incidents. *Mesopotamian Journal of Artificial Intelligence in Healthcare*, *2024*, 184-203. https://doi.org/10.58496/MJAIH/2024/019

electronic health records, which could undermine the confidentiality and integrity of patient data and can hinder access to healthcare systems.[11] The more sensitive data that is collected, the more these AI models become an attractive target for malicious actors.[12] From 2016 to 2021, 374 ransomware attacks on U.S. healthcare organizations exposed the personal health information of 42 million patients, with incidents doubling annually.[13]

The limited safeguards on AI models may incentivize AI developers to use irresponsible data collection and storage practices. Collection limitations are needed.

A key feature of large-scale AI systems is that they are very expensive to develop and run.[14,15] In light of these market pressures, developers may not prioritize investments in privacy and may engage in risky or unsound data practices. For example, there are very limited safeguards surrounding the sensitive data collected by AI-powered "therapists" – and this very sensitive user chat history may be stored for extended periods and used to train other models.[16,17,18] Since complex AI models require immense amounts of data to evolve and improve, developers have little incentive to minimize the collection and use of even the most intimate patient information.

The limited safeguards on AI models may incentivize organizations to use data collected by these models to maximize profit. Use limitations are needed.

Currently, there is limited regulation dictating how organizations can use the patient data that AI collects. This can expose patients to exploitation, since organizations may use this data to maximize profit. For example, it has been well-documented that health insurance plans have been using patient data to generate predictions of medical needs and lengths of stay - irrespective of individual mitigating circumstances – and these predictions are then used to deny claims and

[11] Sallam, M., Al-Mahzoum, K., & Sallam, M. (2024). Generative artificial intelligence and cybersecurity risks: Implications for healthcare security based on real-life incidents. *Mesopotamian Journal of Artificial Intelligence in Healthcare, 2024*, 184–203. https://doi.org/10.58496/MJAIH/2024/019

[12] Kak, A. (2025). *Prepared testimony and statement for the record: "The need to protect Americans' privacy and the AI accelerant" before the U.S. Senate Committee on Commerce, Science, and Transportation.* AI Now Institute. https://www.commerce.senate.gov/services/files/35C1CE13-F03D-4CE9-9559-B0651F156D86

[13] Neprash, H. T., McGlave, C. C., Cross, D. A., et al. (2022). Trends in ransomware attacks on US hospitals, clinics, and other health care delivery organizations, 2016–2021. *JAMA Health Forum, 3*(12), e224873. https://doi.org/10.1001/jamahealthforum.2022.4873

[14] Henshall, W. (2024, June 3). *The billion-dollar price tag of building AI. Time.* https://time.com/6984292/cost-artificial-intelligence-compute-epoch-report/

[15] Kak, A. (2025). *Prepared testimony and statement for the record: "The need to protect Americans' privacy and the AI accelerant" before the U.S. Senate Committee on Commerce, Science, and Transportation.* AI Now Institute. https://www.commerce.senate.gov/services/files/35C1CE13-F03D-4CE9-9559-B0651F156D86

[16] Patel, N. (2025, October 30). *The risks of using patient and provider chat histories to train AI—and how to mitigate them. Health Affairs Forefront.* https://doi.org/10.1377/forefront.20251028.618832

[17] Coghlan, S., Leins, K., Sheldrick, S., Cheong, M., Gooding, P., & D'Alfonso, S. (2023). To chat or bot to chat: Ethical issues with using chatbots in mental health. *Digital health*, *9*, 20552076231183542. https://doi.org/10.1177/20552076231183542

[18] Seigel, R., Augenstein, J., Shashoua, M., Slater, C., & Irlbeck, C. (2025). *Manatt Health: Health AI policy tracker.* Manatt, Phelps & Phillips, LLP. https://www.manatt.com/insights/newsletters/health-highlights/manatt-health-health-ai-policy-tracker

shorten hospital visits.[19] The use of AI models has made it especially difficult for patients to challenge these care denials, since the AI technology used is often proprietary, and there is limited transparency for the AI algorithms used.[20]

There is a lack of transparency regarding when AI is being used.

Even when AI is considered "assistive", the lack of transparency about the AI model and its use can create risk. Clinicians may not always know when outputs are AI-generated, and patients may not know when AI is used in messaging, triage, or documentation.

Additionally, patients may not even be aware that they are "opting in" to use AI models and for their data to be stored and used indefinitely. There are also concerns with patients' ability to "opt in" to the use of their data.[21] Even if a patient "opts in" to share their information for a specific purpose, there is no guarantee that the data will not be used for purposes outside the parameters to which they originally agreed to.[22]

Bias in AI models can deprioritize and discriminate against specific patient groups.

It has been well-documented that AI algorithms inherit, amplify, and perpetuate existing healthcare disparities present in training data, such as underrepresentation of minority groups and unequal care quality.[23, 24] This leads to inherent bias in AI models, which can impact triage, routing, care denials, coding, and documentation. These biases have real-world implications and can worsen health outcomes; for example, AI algorithms have required Black patients to be

---

[19] Ross, C., & Herman, B. (2023, March 13). *Denied by AI: How Medicare Advantage plans use algorithms to cut off care for seniors in need.* STAT News. https://www.statnews.com/2023/03/13/medicare-advantage-plans-denial-artificial-intelligence/

[20] Osman, M., Paul, E., & Weil, E. (2025, October 21). *Calculated need: Algorithms and the fight for Medicaid home care: A comparison of five states' eligibility algorithms.* Upturn. https://www.upturn.org/work/calculated-need/

[21] McNair, D., & Price, W. N., II. (2022). Health care artificial intelligence: Law, regulation, and policy. In the National Academy of Medicine, *Artificial intelligence in health care: The hope, the hype, the promise, the peril* (pp. 167–194). National Academies Press. https://www.nationalacademies.org/read/27111/chapter/9

[22] Kondrup, E. (2025, April 11). *Informed consent, redefined: How AI and big data are changing the rules.* Petrie-Flom Center to Health Law Policy, Biotechnology, and Bioethics, Harvard Law School, Bill of Health Blog. https://petrieflom.law.harvard.edu/2025/04/11/informed-consent-redefined-how-ai-and-big-data-are-changing-the-rules/

[23] Cross, J. L., Choma, M. A., & Onofrey, J. A. (2024). Bias in medical AI: Implications for clinical decision-making. *PLOS Digital Health, 3*(11), e0000651. https://doi.org/10.1371/journal.pdig.0000651

[24] Ranard, B. L., Park, S., Jia, Y., Zhang, Y., Alwan, F., Celi, L. A., & Lusczek, E. R. (2024). Minimizing bias when using artificial intelligence in critical care medicine. *Journal of critical care*, *82*, 154796. https://doi.org/10.1016/j.jcrc.2024.154796

considerably sicker to qualify for support than their White counterparts, and other AI algorithms have proposed inferior medical treatments for patients of minority descent. [25,26,27]

When AI errors occur, accountability is unclear.

Non-medical device AI frequently impacts care directly, as it is used to draft clinical notes, prioritize patient messages or documentation, suggest coding, surface "high risk" patients, and more. When AI informs decisions and errors occur, it is unclear whether accountability should rest on the AI vendor/company, the clinician, and/or the healthcare organization. Although the tool is positioned as "administrative support", its outputs can shape clinical judgement, documentation quality, and downstream care decisions – and accountability mechanisms are needed. To help create these accountability mechanisms, organizations need standardized requirements for: 1) human oversight expectations, 2) documenting when AI was used, and 3) how to handle AI model updates and performance drifts.

What HHS can do

- **HHS can protect patient privacy by issuing practical guidance on minimum privacy safeguards for non-medical device clinical AI**. As HHS considers its role in regulating AI in clinical care, it should emphasize patient privacy and data confidentiality to avoid the improper exposure of sensitive health information. Entities using AI models should have a proactive obligation to place reasonable limits on the collection, use, and retention of personal data, also known as privacy by design. Minimum privacy safeguards should emphasize:
    - *Data minimization in all uses of AI in clinical care and clear data retention limits*. These are some of the best practices to protect patient privacy.
    - *Clear use limitations and default restrictions on secondary use of data.*
    - *Inclusion of a standard "AI Data Use Disclosure" form for clinical settings* that discloses how and when patient data is used, whether data is used for training, and how long it is retained. When AI models are used, this should be clear and transparent, and should *allow patients to opt out of algorithmic decision-making.*
    - *Use of secure reference architectures to reduce data exposure (e.g., secure enclaves, tokenization, local processing of data or community processing, etc.).*[28] Implementation grants can be provided to under-resourced and safety-net providers to fund this architecture.

[25] Obermeyer, Z., Powers, B., & Mullainathan, S. (2019). Dissecting racial bias in an algorithm used to manage the health of populations. *Science, 366*(6464), 447–453. https://doi.org/10.1126/science.aax2342
[26] Bouguettaya, A., Stuart, E. M., & Aboujaoude, E. (2025). Racial bias in AI-mediated psychiatric diagnosis and treatment: A qualitative comparison of four large language models. *NPJ Digital Medicine, 8*(1), Article 332. https://doi.org/10.1038/s41746-025-01746-4
[27] Cedars-Sinai. (2025, June 30). *Cedars-Sinai study shows racial bias in AI-generated treatment regimens for psychiatric patients.* https://www.cedars-sinai.org/newsroom/cedars-sinai-study-shows-racial-bias-in-ai-generated-treatment-regimens-for-psychiatric-patients/
[28] Department of Health and Human Services (2024). HIPAA Security Rule Notice of Proposed Rulemaking to Strengthen Cybersecurity for Electronic Protected Health Information. https://www.hhs.gov/hipaa/for-professionals/security/hipaa-security-rule-nprm/factsheet/index.html

- **HHS should publish a non-medical device clinical AI "governance" playbook that clarifies the minimum expectations for human oversight over the AI, override and stop-use criteria, documentation/audit logging, and change control for model update (including when re-validation/re-evaluation is needed)**. HHS can develop model procurement or contracting clauses that address these performance monitoring responsibilities, especially for smaller and safety net providers that lack negotiating leverage.
- **HHS should establish minimum transparency expectations for non-medical device AI used in clinical care**. Minimum expectations for transparency should include intended use, data sources, known failure modes, policies surrounding AI model updates, and a monitoring plan. There should also be transparency regarding auditable logs for quality review and incident investigations.
- **HHS should provide a pragmatic framework for impact assessment and civil rights assessment.**
  To ensure accountability, HHS should create an actionable framework to guide impact assessments to determine if privacy standards are met, whether particular groups are harmed, and for real consequences for a failure to mitigate or prevent harms.

Question #4: For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful (e.g., contracts, grants, cooperative agreements, and/or prize competitions)?

Experts widely agree that robust evaluation of AI in healthcare is critical to ensuring that the technology is used responsibly. Yet a 2024 scoping review of evaluation methods for clinical AI found "relative scarcity of evidence and guidance for specific practical implementation of performance monitoring of clinical AI." [29] The review found that most clinical AI evaluation metrics focus on accuracy, with other, less common evaluation metrics including assessments of discrimination and calibration (matching AI estimates to real-world outcomes).[30]

Emerging evaluation methods for non-medical device clinical AI

Since the 2024 scoping review, at least two promising methods have been published for non-medical device clinical AI: 1) Stanford's MedHELM (Holistic Evaluation of Language Models) [31] and 2) Elsevier's five-domain evaluation framework (originally created to assess the ClinicalKey generative AI).[32] Both methods have been published in peer-reviewed journals and have been tested on non-medical device generative AI in clinical settings.

---

[29] Andersen, E. S., Birk-Korch, J. B., Hansen, R. S., Fly, L. H., Röttger, R., Arcani, D. M. C., Brasen, C. L., Brandslund, I., & Madsen, J. S. (2024). Monitoring performance of clinical artificial intelligence in health care: a scoping review. *JBI evidence synthesis*, 22(12), 2423–2446. https://doi.org/10.11124/JBIES-24-00042
[30] *Id*.
[31] Bedi, S., Cui, H., Fuentes, M. et al. (2026). Holistic evaluation of large language models for medical tasks with MedHELM. *Nat Med*. https://doi.org/10.1038/s41591-025-04151-2
[32] Livingston, L., Featherstone-Uwague, A., Barry, A., Barretto, K., Morey, T., Herrmannova, D., & Avula, V. (2025). Reproducible generative artificial intelligence evaluation for health care: a clinician-in-the-loop approach. *JAMIA Open*, Volume 8, Issue 3. https://doi.org/10.1093/jamiaopen/ooaf054

Stanford's MedHELM, developed by Stanford Human-Centered AI Center for Research on Foundation Models, assesses AI model performance in 120 healthcare scenarios across 22 categories of health-related tasks, including those related to clinical decision support, clinical note generation, patient communication and education, medical research assistance, and administration and workflow.[33] The MedHELM framework has been tested on six large language models of varying sizes, including OpenAI, Llama, Gemini, and others.[34] Overall, larger AI models did well on complex reasoning tasks (e.g., performing medical calculations and in detecting race bias in clinical text), medium models performed competitively on medical prediction tasks with lower computational demands (e.g., predicting readmission risk), and small models, while adequate for well-structured tasks, struggled with tasks requiring content expertise (particularly in mental health counseling and medical knowledge assessment).[35]

Elsevier's framework assesses generative clinical AI models across five dimensions: 1) query comprehension, 2) response helpfulness, 3) response correctness, 4) response completeness, and 5) potential clinical harm.[36,37] Elsevier originally developed this framework to assess ClinicalKey AI – a generative AI that summarizes medical content to help providers make informed decisions at point-of-care.[38,39] The framework is unique in applying standardized rating scales across the five dimensions, as well as including a robust subject matter expert agreement protocol (to reduce individual subjectivity when assessing AI's response).

Key considerations in AI evaluations

There are several key considerations that are needed in any evaluation of non-medical device clinical AI:

1) **Transparency is needed at all stages of AI model evaluation.** The National Academy of Medicine suggests several forms of transparency are needed, including: 1) disclosure of where training data came from and how it was processed, 2) how the algorithm was developed and validated, 3) how the algorithm reaches its results, and 4) how the pieces of an AI system work together.[40]

---

[33] Center for Research on Foundation Models. (n.d.). *MedHELM: Holistic Evaluation of Large Language Models for Medical Tasks.* Stanford CRFM. https://crfm.stanford.edu/helm/medhelm/latest/#/

[34] Armitage, H. (2025, April 8). *Evaluating AI in context: Which LLM is best for real health care needs?* Stanford Medicine. https://med.stanford.edu/news/insights/2025/04/ai-artificial-intelligence-evaluation-algorithm.html

[35] Shah, N., Pfeffer, M., & Liang, P. (2025, February 28). *Holistic Evaluation of Large Language Models for Medical Applications*. Stanford University Human-Centered Artificial Intelligence. https://hai.stanford.edu/news/holistic-evaluation-of-large-language-models-for-medical-applications

[36] Evans, I. (2025, September 9). *AI Evaluation in Clinical Decision Support: Building a Foundation of Trust*. Elsevier. https://www.elsevier.com/connect/ai-evaluation-in-clinical-decision-support-building-a-foundation-of-trust

[37] Elsevier. (2025). *ClinicalKey AI: Evaluation framework for generative AI tools used for clinical decision support.* https://assets.ctfassets.net/zlnfaxb2lcqx/tenR2MwLZzHVbrMUS1lAR/7d3a049ffc24da170913041323b01f78/ClinicalKey_AI_Evaluation_Framework_WP_2025.pdf

[38] Evans, I. (2025, September 9). *AI Evaluation in Clinical Decision Support: Building a Foundation of Trust*. Elsevier. https://www.elsevier.com/connect/ai-evaluation-in-clinical-decision-support-building-a-foundation-of-trust

[39] Elsevier. (2025). *ClinicalKey AI: Evaluation framework for generative AI tools used for clinical decision support.* https://assets.ctfassets.net/zlnfaxb2lcqx/tenR2MwLZzHVbrMUS1lAR/7d3a049ffc24da170913041323b01f78/ClinicalKey_AI_Evaluation_Framework_WP_2025.pdf

[40] Whicher, D., Ahmed, M., Israni, S.T., et al. (2023). Health Care Artificial Intelligence: Law, Regulation, and Policy. In National Academy of Medicine, *Artificial Intelligence in Health Care: The Hope, the Hype, the Promise, the Peril*. National Academies Press. https://www.ncbi.nlm.nih.gov/books/NBK605945/

2) **Assessment of unintended consequences is essential.** Frameworks recommend systematic monitoring to catch errors and unintended consequences before they cause widespread harm.[41,42,43] Regular testing is important to identify and minimize issues like AI hallucinations, which often go unnoticed by clinicians and can undermine the integrity of a medical record.[44] Continuous monitoring of model performance is also important to measure "model drift"—a phenomenon that occurs when changes in underlying data and relationships between variables degrade the performance of AI models over time.[45] Even small amounts of model drift in reimbursement models could alter the care of hundreds of thousands of enrollees (e.g., changes in socioeconomic factors could cause a model to issue more denials).[46]

3) **Acknowledging and detecting bias in AI models.** It is essential that evaluation protocols are designed to acknowledge and detect bias in clinical AI models. However, experts emphasize that ***eliminating bias in AI clinical care models can be nearly impossible***, because algorithms inherit, amplify, and perpetuate existing healthcare disparities present in training data, such as underrepresentation of minority groups and unequal care quality.[47,48] There is often limited availability of clinical data for groups across certain races/ethnicities, geographic locations, and medical conditions, causing skewed data. While developers should aim to train clinical AI models with data that is representative of the intended patient population (including characteristics such as age, gender, sex, race, ethnicity, geographical location, and medical condition),[49] developers will likely need to implement weighting and/or sampling techniques that attempt to

---

[41] Henry, T. A. (2025, September 4). *Your organization has rolled out the health AI tool. What's next?* AMA. https://www.ama-assn.org/practice-management/digital-health/your-organization-has-rolled-out-health-ai-tool-what-s-next

[42] Adams, K., & Buffett, A. (2025, October 22). *AI and Medicaid: Balancing the Promise of Efficiency with Guardrails to Ensure Responsible Use.* Bipartisan Policy Center. https://bipartisanpolicy.org/article/ai-and-medicaid-balancing-the-promise-of-efficiency-with-guardrails-to-ensure-responsible-use/

[43] Cresswell, K., de Keizer, N., Magrabi, F., Williams, R., Rigby, M., Prgomet, M., Kukhareva, P., Wong, Z. S., Scott, P., Craven, C. K., Georgiou, A., Medlock, S., Brender McNair, J., & Ammenwerth, E. (2024). Evaluating Artificial Intelligence in Clinical Settings-Let Us Not Reinvent the Wheel. *Journal of medical Internet research*, 26, e46407. https://doi.org/10.2196/46407

[44] Deswal, P. (2024, August 7). *Hallucinations in AI-generated medical summaries remain a grave concern*. Clinical Trials Arena. https://www.clinicaltrialsarena.com/news/hallucinations-in-ai-generated-medical-summaries-remain-a-grave-concern/

[45] Nelson, S. D. (2025). *When AI Goes Astray: Understanding Model Drift*. ASHP. https://www.ashp.org/-/media/assets/pharmacy-practice/resource-centers/Digital-Health-and-Artificial-Intelligence/docs/When-AI-Goes-Astray.pdf

[46] Wong, A., & Sussman, J. B. (2025). Understanding Model Drift and Its Impact on Health Care Policy. *JAMA Health Forum*. 6(8):e252724. https://jamanetwork.com/journals/jama-health-forum/fullarticle/2837524

[47] Cross, J. L., Choma, M. A., & Onofrey, J. A. (2024). Bias in medical AI: Implications for clinical decision-making. *PLOS Digit Health*. 3(11):e0000651. https://pmc.ncbi.nlm.nih.gov/articles/PMC11542778/

[48] Ranard, B. L., Park, S., Jia, Y., Zhang, Y., Alwan, F., Celi, L. A., & Lusczek, E. R. (2024). Minimizing bias when using artificial intelligence in critical care medicine. *Journal of critical care*, 82, 154796. https://doi.org/10.1016/j.jcrc.2024.154796

[49] Artificial Intelligence/Machine Learning-enabled Working Group. (2025, January 27). *Good machine learning practice for medical device development: Guiding principles.* International Medical Device Regulators Forum. https://www.imdrf.org/sites/default/files/2025-02/IMDRF_AIML%20WG_GMLP_N88%20Final.pdf

balance the data so it does not favor the majority group.[50] Researchers can test for statistical parity to determine whether the model provides equal responses across different protected groups,[51] and regular monitoring of AI models should include impact assessments that evaluate whether particular groups face higher rates of unintended consequences or harm.

What HHS can do

- HHS can require extensive *transparency* regarding the AI used in clinical settings – this transparency is essential to support effective AI evaluations. As identified by NIH, transparency should include how the AI tool was developed (data used to develop the tool, how the algorithm was developed and validated, how the algorithm reaches its results, etc.), as well as how the tool is being used in practice.[52]
- HHS can require that evaluation methods that include metrics of "responsible" use of AI in healthcare, such as: [53]
  1) the extent of human oversight
  2) complete disclosure as to how systems work, data used, and appeal processes
  3) the presence of small-scale testing to catch errors before they cause widespread harm
  4) efforts to protect the most vulnerable, including extra safeguards for children, people with disabilities, and non-English speakers
  5) demonstrating that AI tools help, not harm
- Although evaluations are essential to reduce harm that AI can cause in clinical settings, HHS can acknowledge that there are areas of clinical care where AI is likely to be more harmful than helpful to patients' health and safety, especially among the most low-income and vulnerable patients.[54] HHS can acknowledge that **AI will never be completely free of errors and bias**, and therefore, there are limitations to using AI for certain clinical tasks, such as determinations of medical necessity and denials of care.
- HHS can accelerate responsible adoption of ethical, transparent non-medical device AI tools by helping the field converge on a shared evaluation infrastructure, and by supporting multi-site learning and demonstration projects of its use across diverse healthcare settings and patient populations. The following are a few example HHS mechanisms that would be most impactful in supporting promising HHS evaluation

---

[50] Norori, N., Hu, Q., Aellen, F. M., Faraci, F. D., & Tzovara, A. (2021). Addressing bias in big data and AI for health care: A call for open science. *Patterns* (New York, N.Y.), 2(10), 100347. https://doi.org/10.1016/j.patter.2021.100347

[51] Chen, F., Wang, L., Hong, J., Jiang, J., & Zhou, L. (2024). Unmasking bias in artificial intelligence: a systematic review of bias detection and mitigation strategies in electronic health record-based models. *Journal of the American Medical Informatics Association* : JAMIA, 31(5), 1172–1183. https://doi.org/10.1093/jamia/ocae060

[52] Whicher, D., Ahmed, M., Israni, S.T., et al. (2023). Health Care Artificial Intelligence: Law, Regulation, and Policy. In National Academy of Medicine, *Artificial Intelligence in Health Care: The Hope, the Hype, the Promise, the Peril*. National Academies Press. https://www.ncbi.nlm.nih.gov/books/NBK605945/

[53] Adams, K., & Buffett, A. (2025, October 22). *AI and Medicaid: Balancing the Promise of Efficiency with Guardrails to Ensure Responsible Use.* Bipartisan Policy Center. https://bipartisanpolicy.org/article/ai-and-medicaid-balancing-the-promise-of-efficiency-with-guardrails-to-ensure-responsible-use/

[54] Techtonic Justice. (2024). *Inescapable AI: The Ways AI Decides How Low-Income People Work, Live, Learn, and Survive*. Techtonic Justice. https://www.techtonicjustice.org/reports/inescapable-ai

methods from pre- and post-deployment metrics, robustness testing, workflow, and human-centered evaluation methods.

1) Grants or cooperative agreements to build a shared evaluation infrastructure and common protocols – that includes at the minimum, benchmark datasets and test cases – including those "edge cases" that frequently drive harm or failure for some patient groups that may not be represented in the data (e.g., missing context, contradictory documentation, rare conditions, multi- or co-morbidity, literacy or language variation).
2) Contracts to stand up multi-site pilot networks across diverse healthcare settings
3) Prize competitions or innovative mechanisms to advance robust evaluation methods, performance drift detection, and fairness under shift (i.e., when population mix changes, documentation patterns change, or language needs differ from when AI was built)
4) Encourage Public-Private partnerships / and a Cooperative Research and Development agreement (CRADA) to create and co-develop standardized governance and evaluation templates that can be adopted to co-develop and field-test practical assets (e.g., evaluation toolkits, benchmarking methods, governance templates) with implementers and vendors in real clinical environments.

Question #6: Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve healthcare outcomes, give new insights on quality, and help reduce costs?

Where AI tools have fallen short: hallucinations/errors, bias, claims denials, and treatment timeline predictions

Hallucinations/errors

A well-documented area where AI tools have fallen short is their tendency to "hallucinate," or invent information and present it as fact.[55] This phenomenon has broad implications, as it may occur in any clinical setting that uses AI systems. A meta-analysis of hallucination incidence in Large Language Model (LLM) responses to oncology questions found that approximately one in five responses contained inaccurate information.[56] LLMs are particularly prone to hallucination when false details are entered into the system, with one study finding LLM models repeat or elaborate on a planted error in up to 83% of cases of clinical decision support.[57] AI hallucinations have also been documented in medical devices, including imaging devices and

[55] Hatem, R., Simmons, B., & Thornton, J. E. (2023). A Call to Address AI "Hallucinations" and How Healthcare Professionals Can Mitigate Their Risks. *Cureus* 15(9): e44720. https://doi.org/10.7759/cureus.44720
[56] Yoon, M. S. (2025). Navigating artificial intelligence (AI) accuracy: A meta-analysis of hallucination incidence in large language model (LLM) responses to oncology questions. Journal of Clinical Oncology. https://doi.org/10.1200/JCO.2025.43.16_suppl.e13686
[57] Omar, M. et al. (2025) Multi-model assurance analysis showing large language models are highly vulnerable to adversarial hallucination attacks during clinical decision support, Communications Medicine. https://doi.org/10.1038/s43856-025-01021-3

generative-based synthetic medical images.[58] This could have devastating consequences for patients: not only could AI hallucinations result in incorrect diagnoses and treatment,[59] but incorrect information entered into a medical record can be propagated across multiple systems and providers, and could create long-term consequences that extend beyond the healthcare setting.[60] In the context of health coverage, hallucinations could result in the inclusion of false information about a patient's diagnosis in an AI-generated prior authorization request, resulting in improper claims adjudication.[61]

Bias

As mentioned earlier, AI algorithms inherit, amplify, and perpetuate existing healthcare disparities present in training data, such as underrepresentation of minority groups and unequal care quality.[62,63] These biases have real-world implications; for example, AI algorithms have required Black patients to be considerably sicker to qualify for support than their White counterparts, and other AI algorithms have proposed inferior medical treatments for patients of minority descent.[64,65,66]

Clinicians are deeply concerned about AI bias, and call for strong oversight frameworks and data transparency to allow for the evaluation of AI models.[67,68,69] While some providers certainly see the value that AI tools may have in clinical spaces, they do not want the integration of these

---

[58] Granstedt, J., Kc, P., Deshpande, R., Garcia, V., & Badano, J. (2025). Hallucinations in medieval devices. *Artificial Intelligence in the Life Sciences*. 8, 100145. https://doi.org/10.1016/j.ailsci.2025.100145
[59] Maleki, N., Padmanabhan B., and Dutta, K. (2024). AI Hallucinations: A Misnomer Worth Clarifying, 2024 IEEE Conference on Artificial Intelligence (CAI), Singapore. https://ieeexplore.ieee.org/abstract/document/10605268
[60] Siwicki, B. (2025). The damage AI Hallucinations can do - and how to avoid them, Healthcare IT News. https://www.healthcareitnews.com/news/damage-ai-hallucinations-can-do-and-how-avoid-them
[61] Shift Technology, (2025). Shift Insurance Perspectives: The AI Hallucinations Edition. https://www.shift-technology.com/resources/reports-and-insights/shift-insurance-perspectives-the-ai-hallucinations-edition
[62] Cross, J.L., Choma, M. A., Onofrey, J. A. (2024). Bias in medical AI: Implications for clinical decision-making. PLOS Digit Health. https://pmc.ncbi.nlm.nih.gov/articles/PMC11542778/
[63] Ranard, B. L., et al. (2024). Minimizing bias when using artificial intelligence in critical care medicine. J Crit Care. National Library of Medicine. https://doi.org/10.1016/j.jcrc.2024.154796
[64] Obermeyer, Z. (2019). Dissecting racial bias in an algorithm used to manage the health of populations, Science. https://www.science.org/doi/10.1126/science.aax2342
[65] Bouguettaya, A., Stuart, E. M., & Aboujaoude, E. (2025). Racial bias in AI-mediated psychiatric diagnosis and treatment: A qualitative comparison of four large language models. *NPJ Digital Medicine, 8*(1), Article 332. https://doi.org/10.1038/s41746-025-01746-4
[66] Cedars Sinai, (2025). Cedars-Sinai Study Shows Racial Bias in AI-Generated Treatment Regimens for Psychiatric Patients. https://www.cedars-sinai.org/newsroom/cedars-sinai-study-shows-racial-bias-in-ai-generated-treatment-regimens-for-psychiatric-patients/
[67] Jain, A., et al. (2023). Awareness of Racial and Ethnic Bias and Potential Solutions to Address Bias With Use of Health Care Algorithms, JAMA Health Forum, JAMA Network. https://jamanetwork.com/journals/jama-health-forum/fullarticle/2805595
[68] Bervell, J. (2023, April 14). Can AI Improve Health Without Perpetuating Bias? The Dose, The Commonwealth Fund. https://www.commonwealthfund.org/publications/podcast/2023/apr/can-ai-improve-health-without-perpetuating-bias
[69] Levi, R., (2023). Rooting Out Racial Bias in Health Care AI, Part 2, Health Technology, Trade Offs. https://tradeoffs.org/2023/06/01/artificial-intelligence-ai-racial-bias-2/

technologies to come at the expense of their commitment to "do no harm," which includes protecting those most vulnerable.[70]

Claims denials

Another well-documented area where AI tools have fallen short is when they are used to process insurance claims. Indeed, investigations of AI systems used by insurance companies reveal that these systems are ***built to deny claims first***: when a claim enters the system, the predisposition (or default) of the algorithm is to deny the claim, rather than accept it or remain neutral in its decision-making.[71] While insurers maintain that humans with medical expertise still review and approve the denials, research has revealed that these reviewers frequently simply approve the denial that has been prepackaged by the AI system – without further investigation or utilizing medical judgment. "'We literally click and submit,' one former Cigna doctor said. 'It takes all of 10 seconds to do 50 at a time.'"[72] In a two-month period, Cigna denied more than 300,000 claims, which amounts to about 1.2 seconds for each physician-reviewed claim.[73] A 2024 survey by the National Association of Insurance Commissioners of large health insurers found 37% of insurers report using AI for prior authorization, 44% for claims adjudication, and 56% for broadly defined utilization management activities.[74]

Treatment timeline predictions

Relatedly, AI tools have fallen short in their ability to accurately assess medical need and treatment timelines – which insurance companies use to deny claims and cut off funding for treatment.[75] A STAT investigation found that Medicare Advantage plans' use of algorithms to make coverage determinations has driven high rates of denials, which have the potential to harm more than 31 million enrollees.[76] Between 2020 and 2022 (when Medicare Advantage plans started using these algorithms), the number of appeals filed to contest MA denials shot up to 58%, which only accounts for denials that were officially appealed.[77] AI models often fail to consider individual circumstances or changes in condition. As a result, patients have been denied

[70] Adams, K., Buffet, A., (2025). AI and Medicaid: Balancing the Promise of Efficiency with Guardrails to Ensure Responsible Use, Bipartisan Policy Center. https://bipartisanpolicy.org/article/ai-and-medicaid-balancing-the-promise-of-efficiency-with-guardrails-to-ensure-responsible-use/
[71] Rucker, P., The Capitol Forum, Miller, M., David Armstrong, (2025, March 23) How Cigna Saves Millions by Having Its Doctors Reject Claims Without Reading Them, Health Care, ProPublica. https://www.propublica.org/article/cigna-pxdx-medical-health-insurance-rejection-claims
[72] *Id.*
[73] Schreiber, M., (2025, Jan. 25). New AI tool counters health insurance denials decided by automated algorithms, The Guardian. https://www.theguardian.com/us-news/2025/jan/25/health-insurers-ai
[74] DeFrain, C., et al. (2025). Health Insurance Artificial Intelligence/Machine Learning Survey Results, NAIC Staff Report, National Association of Insurance Commissioners. https://content.naic.org/sites/default/files/inline-files/NAIC%20AI%20Health%20Survey%20Report%20.pdf
[75] Mello, M., et al., (2026). The AI Arms Race In Health Insurance Utilization Review: Promises Of Efficiency And Risks Of Supercharged Flaws, Vol. 45, No. 1: Artificial Intelligence, Health Spending & More, Health Affairs. https://doi.org/10.1377/hlthaff.2025.00897
[76] Ross, C., Herman, H., (2023). Denied by AI: How Medicare Advantage plans use algorithms to cut off care for seniors in need, STAT. https://www.statnews.com/2023/03/13/medicare-advantage-plans-denial-artificial-intelligence/
[77] *Id.*

rehabilitation stays, discharged with severe pain, and have developed complications due to lack of care continuity.[78]

Where AI tools have been of assistance: detecting certain diseases early and accurately, and assisting with medical documentation

Detecting certain diseases early and accurately

There are several areas where AI tools have proven value in clinical settings. Research demonstrates that AI has the ability to detect rare diseases earlier and with better accuracy, including better interpretations of imaging results. One study of AI functions in diagnostic imaging showed algorithms can analyze medical images, such as X-rays, CT scans, and MRIs, with greater accuracy and speed than human radiologists.[79] With access to medical history through integration in EHRs, AI tools are able to identify patterns and often detect diseases like cancer at earlier stages.[80,81] This helps reduce misdiagnoses and ensures patients receive correct treatment promptly.

Assisting with medical documentation

AI also has the potential to streamline certain documentation processes, allowing clinical staff more time to focus on patient care. A 2021 study conducted by the American Academy of Family Physicians found that the adoption of an AI assistant for documentation reduced the median documentation time per note by 72% for family physicians and PCPs.[82] Reducing the time physicians spend interacting with electronic health records not only frees up time for providers to deliver direct care to patients, but it is also associated with reduced clinician burnout.[83] States have also adopted automation and machine learning tools for this purpose: in Ohio, for example, the "Baby Bot" automatically adds newborns to their mother's Medicaid case, saving weeks' worth of county staff time.[84]

What HHS can do

- HHS can put clear guardrails around how AI is used in clinical care to address the real harm that arises when AI is used to issue claims denials and determine treatment timelines. For example, HHS should prioritize transparency by requiring insurers to

---

[78] *Id.*
[79] Khalifa, M., Albadawy, M., (2024). AI in diagnostic imaging: Revolutionising accuracy and efficiency, Volume 5, Science Direct. https://doi.org/10.1016/j.cmpbup.2024.100146
[80] *Id.*
[81] Papreddy, P., et. al. (2025). Transforming sepsis management: AI-driven innovations in early detection and tailored therapies, National Library of Medicine. https://pmc.ncbi.nlm.nih.gov/articles/PMC12366378/
[82] American Academy of Family Physicians. (2021, November). *Using an AI assistant to reduce documentation burden in family medicine: Evaluating the Suki Assistant* (Innovation Labs report). https://www.tafp.org/media/healthy-practice/report-suki-assistant-documentation-burden.pdf
[83] Pearlman, K., Wan, W., Shah, S., & Laiteerapong, N. (2025). Use of an AI scribe and electronic health record efficiency. *JAMA Network Open*, *8*(10), e2537000. https://doi.org/10.1001/jamanetworkopen.2025.37000
[84] Adams, K., & Buffett, A. (2025, October 22). *AI and Medicaid: Balancing the promise of efficiency with guardrails to ensure responsible use*. Bipartisan Policy Center. https://bipartisanpolicy.org/article/ai-and-medicaid-balancing-the-promise-of-efficiency-with-guardrails-to-ensure-responsible-use/

notify patients when insurers have used AI to issue their claims decisions. HHS also could ban insurers from using AI when determining medical necessity. Alternatively, HHS could direct insurers to include sufficient and thorough human oversight by requiring that ultimate claims decisions be made by a qualified human professional. HHS can also ensure that patients have access to a meaningful and easy way to navigate systems to challenge claim denials that involve a qualified human reviewer who was not involved in the initial decision.

Question #9: What challenges within healthcare do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

Concerns patients and caregivers have related to the adoption of AI in clinical care

While certain groups of patients are open to the integration of AI in clinical spaces, broadly speaking, patients are still skeptical about AI tools. A 2023 Pew Research Center survey of U.S. adults revealed Americans have little trust in companies to use AI responsibly:[85]

- Most Americans (70%) say they have little or no trust in companies to make responsible decisions about how they use AI in their products;
- 81% say information collected will be used in ways people are not comfortable with;
- 80% say it will be used in ways that were not originally intended.

Caregivers are especially concerned about the use of AI algorithms to determine "medical necessity" and make claim denials. The use of AI models has made it especially difficult for caregivers and their care recipients to challenge these care denials because they are opaque - the AI technology used is often proprietary, and limited transparency means they cannot understand why they were denied care.[86] Caregivers of patients who require Home and Community-Based Services (HCBS) are particularly concerned that AI algorithms are not designed to identify people's real needs, but rather are used as tools to limit access to HCBS by applying restrictive eligibility criteria.[87] If patients and their caregivers cannot access the HCBS they need, this can hinder patients' quality of life while placing additional burden on already overstrained caregivers.[88]

There are also concerns about the erosion of the patient-provider relationship and loss of face-time with clinicians, as providers have a growing reliance on AI tools. The Association of American Medical Colleges warns that AI may serve as a substitute for human connection, which can undermine patient-provider trust and may lead to subsequent issues with patient treatment compliance.[89] Moreover, AI cannot determine which aspects of a patient's life are

---

[85] Faverio, M. (2023, October 18). Key findings about Americans and data privacy. Pew Research Center. https://www.pewresearch.org/short-reads/2023/10/18/key-findings-about-americans-and-data-privacy/
[86] Osman, M., Paul, E., & Weil, E. (2025, October 21). *Calculated need: Algorithms and the fight for Medicaid home care*. Upturn. https://www.upturn.org/work/calculated-need/
[87] *Id.*
[88] *Id.*
[89] Boyle, P. (2025, November 2). Doctors, beware: AI threatens to weaken your relationships with patients. Association of American Medical Colleges. https://www.aamc.org/news/doctors-beware-ai-threatens-weaken-your-relationships-patients

clinically relevant: for example, a physician treating a patient with diabetes may know it is important to ask whether the patient is still living with a family member who brings them to the grocery store, but an AI tool may overlook the clinical significance of this exchange.[90]

Challenges within healthcare that patients and caregivers wish to see addressed by AI in clinical care

More generally, US adults are interested in and most comfortable with AI innovations that provide recommendations of providers or care settings, ease their caregiving burdens, assist with diagnoses (leveraged and reviewed by their doctor), and assist with clinical administrative tasks.[91] Among adult age groups, Generation X and Millennials report the most interest and use of AI in clinical care.[92]

What HHS can do

As HHS develops AI policies regarding patient rights, transparency, and guardrails, HHS can clearly and thoroughly address some of the most common patient and caregiver concerns in its policies. The most common patient and caregiver concerns that can be addressed include:

- Privacy: "Who sees my data? Is it training a model?"
- Transparency: "Is AI being used on me? Can I opt out?"
- Errors/hallucinations: "What is AI is wrong? Who catches it?"
- Appeals/recourse: "How do I challenge AI-driven decisions?"
- Bias and unequal quality: "Will it work as well for me as others?"
- Loss of human care: "Will this reduce time with my clinician?"

Question #10: Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care? Are there published findings about the impact of adopted AI tools and their use clinical care? How does the literature approach the costs, benefits, and transfers of using AI as part of clinical care?

Below, we summarize three well-documented impacts of the adoption of AI tools in clinical care. Adopting AI tools in clinical care has been shown to 1) improve efficacy in the diagnosis of select health conditions, 2) reduce provider burnout when leveraged for medical documentation, and 3) lead to algorithmic discrimination.

1) Adopting AI tools in clinical care may improve efficiency in the diagnosis of select health conditions

A growing body of literature suggests AI may improve efficiency in diagnosing certain health conditions, especially those that require radiology diagnoses. A 2023 review confirmed that AI enhances the radiology process by both automating triage (AI can find and highlight critical

---

[90] *Id.*

[91] PricewaterhouseCoopers. (2025, October 20). PwC's 2025 US Healthcare Consumer Insights Survey: The consumer-first era of healthcare. PwC. https://www.pwc.com/us/en/industries/health-industries/library/healthcare-consumer-insights-survey.html

[92] *Id.*

cases for immediate review) and generating and interpreting high-quality radiology reports.[93] As a result, as of July 2025, the FDA has authorized over 1,250 AI-enabled medical devices for marketing in the United States, with 77% of authorized devices designed for use in radiology.[94] AI tools have been successful in diagnosing numerous other specific health conditions, such as diabetic retinopathy, atrial fibrillation, and coronavirus pneumonia.[95,96,97] Indeed, the FDA approved an autonomous AI-based diagnostic tool for diabetic retinopathy – without the need for specialist interpretation and oversight – due to the model's superior detection of disease indicators in medical images.[98]

2) Adopting AI tools for medical documentation can reduce provider burnout

The use of AI in clinical settings has the potential to decrease providers' administrative burdens and thereby reduce burnout. Numerous studies have indicated that AI models can successfully act as "scribes" and reduce the time providers engage in medical documentation. A 2021 study conducted by the American Academy of Family Physicians found that the adoption of an AI assistant for documentation reduced the median documentation time per note by 72% for family physicians and PCPs.[99] One study found that clinicians across six health systems using ambient AI scribes to document patient-clinician conversations reported a decrease in burnout rates from 51.9% to 38.8% after one month;[100] They also reported reduced cognitive load, less time spent on after-hours documentation, and improved focus on patient interactions.[101] Of physicians polled in the AMA's 2024 AI Physician Sentiment Survey, 57% viewed addressing

[93] Najjar, R. (2023). *Redefining radiology: A review of artificial intelligence integration in medical imaging*. *Diagnostics, 13*(17), 2760. https://doi.org/10.3390/diagnostics13172760
[94] Sandalow, M., Adams, K., & Loud, G. (2025, November 10). FDA oversight: Understanding the regulation of health AI tools (Issue Brief). Bipartisan Policy Center. https://bipartisanpolicy.org/issue-brief/fda-oversight-understanding-the-regulation-of-health-ai-tools/
[95] Abràmoff, M. D., Lavin, P. T., Birch, M., Shah, N., & Folk, J. C. (2018). Pivotal trial of an autonomous AI-based diagnostic system for detection of diabetic retinopathy in primary care offices. NPJ Digital Medicine, 1, 39. https://doi.org/10.1038/s41746-018-0040-6
[96] Lin, F., Zhang, P., Chen, Y., Liu, Y., Li, D., Tan, L., Wang, Y., Wang, D. W., Yang, X., Ma, F., & Li, Q. (2024). Artificial-intelligence-based risk prediction and mechanism discovery for atrial fibrillation using heart beat-to-beat intervals. Med, 5(5), 414–431.e5. https://doi.org/10.1016/j.medj.2024.02.006
[97] Zhang, K., Liu, X., Shen, J., Li, Z., Sang, Y., Wu, X., Zha, Y., Liang, W., Wang, C., Wang, K., Ye, L., Gao, M., Zhou, Z., Li, L., Wang, J., Yang, Z., Cai, H., Xu, J., Yang, L., … Wang, G. (2020). Clinically applicable AI system for accurate diagnosis, quantitative measurements, and prognosis of COVID-19 pneumonia using computed tomography. Cell, 181(6), 1423–1433.e11. https://doi.org/10.1016/j.cell.2020.04.045
[98] Abràmoff, M. D., Lavin, P. T., Birch, M., Shah, N., & Folk, J. C. (2018). Pivotal trial of an autonomous AI-based diagnostic system for detection of diabetic retinopathy in primary care offices. NPJ Digital Medicine, 1(1), 39. https://doi.org/10.1038/s41746-018-0040-6
[99] American Academy of Family Physicians. (2021, November). Using an AI assistant to reduce documentation burden in family medicine: Evaluating the Suki Assistant (Innovation Labs report). https://www.tafp.org/media/healthy-practice/report-suki-assistant-documentation-burden.pdf
[100] Olson, K. D., Meeker, D., Troup, M., Barker, T. D., Nguyen, V. H., Manders, J. B., Stults, C. D., Jones, V. G., Shah, S. D., Shah, T., & Schwamm, L. H. (2025). Use of ambient AI scribes to reduce administrative burden and professional burnout. JAMA Network Open, 8(10), e2534976. https://doi.org/10.1001/jamanetworkopen.2025.34976
[101] *Id.*

administrative burden through automation as the biggest area of opportunity for AI, and most physicians believed AI tools would be helpful in alleviating stress and burnout.[102]

3) Algorithmic discrimination is "baked into" AI models, which harms the most low-income and vulnerable populations.

While there are documented benefits to some uses of AI in clinical settings, these tools are not without their limitations. One serious, well-documented harm is "algorithmic discrimination," which occurs when AI systems make prejudicial decisions based on attributes like race, gender, age, sexual orientation, or other demographic characteristics. As mentioned earlier, there exists significant evidence of racial bias in clinical AI, as these tools pick up and perpetuate bias from the data they are trained on and the human interactions they learn from.[103] Because racial and ethnic minority groups are underrepresented in medical research, this can lead to worsened diagnostic accuracy and treatment recommendations for these groups.[104,105] This bias is nearly impossible to address in AI models.

Algorithmic discrimination has real consequences for patient care: a 2019 landmark study found a widely used algorithm designed to identify patients for high-risk care management programs required Black patients to be considerably sicker to qualify for support than their White counterparts.[106] The algorithm was designed to predict healthcare costs rather than illness, but failed to account for unequal access to care and resulting spending differences between racial groups.[107] If the disparity was remedied, the percentage of Black patients receiving additional support would have increased from 17.7% to 46.5%.[108] In another example, a 2025 study of LLMs used in psychiatry found the LLMs assumed racial identity from patient names or other data and proposed inferior treatments for patients of minority descent.[109,110]

---

[102] American Medical Association. (2025, February). Physician sentiments around the use of AI in health care: Motivations, opportunities, risks, and use cases (Shifts from 2023 to 2024) (AMA Augmented Intelligence Research Report). https://www.ama-assn.org/system/files/physician-ai-sentiment-report.pdf
[103] Hurd, T. C., Cobb Payton, F., & Hood, D. B. (2024). Targeting machine learning and artificial intelligence algorithms in health care to reduce bias and improve population health. The Milbank Quarterly, 102(3), 577–604. https://doi.org/10.1111/1468-0009.12712
[104] Rutgers University–Newark. (2024, December 1). AI algorithms used in healthcare can perpetuate bias. https://www.newark.rutgers.edu/news/ai-algorithms-used-healthcare-can-perpetuate-bias
[105] Hanna, M. G., Pantanowitz, L., Jackson, B., Palmer, O., Visweswaran, S., Pantanowitz, J., Deebajah, M., & Rashidi, H. H. (2025). Ethical and bias considerations in artificial intelligence/machine learning. Modern Pathology, 38(3), 100686. https://doi.org/10.1016/j.modpat.2024.100686
[106] Obermeyer, Z., Powers, B., Vogeli, C., & Mullainathan, S. (2019). Dissecting racial bias in an algorithm used to manage the health of populations. Science, 366(6464), 447–453. https://doi.org/10.1126/science.aax2342
[107] *Id.*
[108] *Id.*
[109] Bouguettaya, A., Stuart, E. M., & Aboujaoude, E. (2025). Racial bias in AI-mediated psychiatric diagnosis and treatment: A qualitative comparison of four large language models. *NPJ Digital Medicine*, *8*(1), Article 332. https://doi.org/10.1038/s41746-025-01746-4
[110] Cedars-Sinai. (2025, June 30). *Cedars-Sinai study shows racial bias in AI-generated treatment regimens for psychiatric patients.* https://www.cedars-sinai.org/newsroom/cedars-sinai-study-shows-racial-bias-in-ai-generated-treatment-regimens-for-psychiatric-patients/

What HHS can do

- HHS must acknowledge that algorithmic discrimination causes real harm and is nearly impossible to avoid or prevent, and therefore must put clear guardrails around how AI is used in clinical care (i.e., AI cannot be used for care determinations and care denials without the involvement of a qualified human professional to review these decisions).
- HHS should encourage and support research that investigates the extent to which AI in clinical care results in actual or potential harm to patients and that develops evidence-based approaches to reduce such harm to the maximum extent possible.

Conclusion

For all the foregoing reasons, the individual public health deans and scholars listed below urge HHS to provide oversight and regulation regarding the use of AI in clinical care. AI has extensive potential for making clinical care more efficient and accessible. However, without regulation and oversight, it also has the strong potential to cause irreparable harm in the context of healthcare.

We also respectfully request that the full text of our comments, as well as the full text of each of the individual studies, reports, and other supporting materials that we have cited and made available through active links in our comments, be considered part of the formal administrative record on this request for information from the HHS for purposes of the Administrative Procedure Act. Please let us know if HHS is unable for any reason to include our linked materials, so we will have the chance to otherwise submit copies of the supporting documents into the administrative record.

Thank you for your consideration of our comments. If you need any additional information, please contact Allyson Crays at a.crays@gwu.edu.

A. Public Health Deans

1. Abroms, Lorien, ScD, Professor, Associate Dean for PhD/MS Programs, Milken Institute School of Public Health, The George Washington University

2. Catalanotti, Jillian, MD, MPH, FACP, Associate Dean for Clinical Public Health & Population Health Practice, Associate Professor of Medicine, Associate Professor of Health Policy and Management, Director, Internal Medicine Residency Programs, The George Washington University

3. Chandler, G. Thomas, MS, PhD, Distinguished Dean Emeritus, Professor, Environmental Health Sciences, Arnold School of Public Health, University of South Carolina

4. El-Mohandes, Ayman, MBBCh, MD, MPH, Dean, CUNY Graduate School of Public Health & Health Policy

5. Goldman, Lynn R., MD, MPH, MS, Dean Emerita, Milken Institute School of Public Health, The George Washington University

6. Gusmano, Michael K., PhD, lacocca Chair, Professor of Health Policy, Associate Dean of Academic Affairs, College of Health, Lehigh University

7. Hyder, Adnan A., MD, MPH, PhD, Dean & Robert A. Knox Professor, Boston University School of Public Health

8. Jeffries, Pamela R., PhD, RN, FAAN, ANEF, FSSH, Dean, Vanderbilt School of Nursing, Valere Potter Distinguished Professor of Nursing, RWJF Nurse Executive Fellow Alumna, Vanderbilt School of Nursing

9. LaVeist, Thomas A., PhD, Dean and Weatherhead Presidential Chair in Health Equity, Professor Tulane University School of Public Health and Tropical Medicine

10. Lu, Michael C., MD, MS, MPH, Dean, UC Berkeley School of Public Health

11. Lushniak, Boris, MD, MPH, Professor and Dean, University of Maryland School of Public Health

12. Rimer, Barbara K., DrPH, Professor Emerita, Department of Health Behavior, Dean Emerita, UNC Gillings School of Global Public Health

13. Thorpe, Jane, JD, Professor and Sr. Associate Dean for Academic, Student & Faculty Affairs, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

14. Vermund, Sten H., MD, PhD, Dean, Distinguished University Health Professor of Public Health and Pediatrics, College of Public Health, University of South Florida

15. Wilensky, Sara E., JD, PhD, Assistant Dean for Undergraduate Education, Director, Undergraduate Program in Public Health, Special Services Faculty for Undergraduate Education, Milken Institute School of Public Health, The George Washington University

B. Health Professions, Public Health, Health Law and Policy Scholars

16. Akman, Jeffrey S., MD, Professor and Leon M. Yochelson Chair, Department of Psychiatry and Behavioral Sciences, School of Medicine and Health Sciences, The George Washington University

17. Baird, Sarah, PhD, Professor and Vice Chair, Department of Global Health, Milken Institute School of Public Health, The George Washington University

18. Barkoff, Alison, JD, Harold and Jane Hirsh Associate Professor of Health Law and Policy, Director, Hirsh Health Law and Policy Program, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

19. Beckerman, Julia Zoe, JD, MPH, Teaching Professor and Vice Chair of Academics, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

20. Blanchard, Janice, MD, PhD, Professor, Chief of Health Policy Section, Department of Emergency Medicine, School of Medicine and Health Sciences, The George Washington University

21. Blatt, Benjamin, MD, CHSE, Professor of Medicine, School of Medicine and Health Sciences, The George Washington University

22. Blewett, Lynn A., PhD, MA, Professor, Division of Health Policy and Management, University of Minnesota School of Public Health

23. Bonar, Robert, DHA, Gordon A. Friesen Professor of Healthcare Administration, Director of Master of Health Administration Program, Milken Institute School of Public Health, The George Washington University

24. Borkowski, Liz, MPH, Senior Research Scientist, Managing Director, Jacobs Institute of Women's Health, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

25. Braaten, Kari P., MD, MPH, Assistant Professor of Obstetrics, Gynecology and Reproductive Biology, Harvard Medical School

26. Braffett, Barbara H., PhD, Associate Research Professor, Department of Epidemiology, Co-PI, DCCT/EDIC Data Coordinating Center, Co-I, TODAY Coordinating Center, The George Washington University

27. Brindis, Claire D., DrPH, Distinguished Professor, Departments of Pediatrics and Obstetrics, Gynecology and Reproductive Sciences, University of California, San Francisco, Emerita Director, Philip R. Lee Institute for Health Policy Studies

28. Burke, Taylor, JD, LLM, Adjunct Professor of Health Law and Policy, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

29. Burroughs, Thomas E., PhD, MS, MA, Professor of Health Management & Policy, Biostatistics, and Epidemiology, College for Public Health and Social Justice, Saint Louis University

30. Byrnes, Maureen, MPA, Teaching Instructor, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

31. Cartwright-Smith, Lara, JD, MPH, Associate Professor, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

32. Casoni, Maria, MPH, MSL, Research Scientist, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

33. Chang, Aileen, MD, MSPH, Associate Professor of Medicine, The George Washington University School of Medicine and Health Sciences

34. Chernoby, Kimberly, MD, JD, Clinical Instructor of Emergency Medicine, The George Washington University School of Medicine and Health Sciences

35. Cleary, Sean, MPH, PhD, Associate Professor, Department of Epidemiology, Milken Institute School of Public Health, The George Washington University

36. Cohen, Alan B., Sc.D., Research Professor (Retired), Markets, Public Policy and Law, Boston University Questrom School of Business, and Professor of Health Law, Policy and Management (Retired), Boston University School of Public Health

37. Crays, Allyson, JD, Public Health Law and Policy Analyst, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

38. Dankwa-Mullan, Irene, MD MPH, Adjunct Professor, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

39. Diemert, David, MD, FRCP(C), Professor, Depts. of Medicine and Microbiology, Immunology & Tropical Medicine, The George Washington University School of Medicine & Health Sciences

40. Dutta, Elizabeth Arend, DrPH, MPH, Professorial Lecturer, The George Washington University

41. Ecker, Jeffrey L., MD, Joe V. Meigs Professor of Obstetrics, Gynecology and Reproductive Biology, Harvard Medical School, Chair Emeritus, Department of Obstetrics and Gynecology, Massachusetts General Hospital

42. Erikson, Clese, MPAff, Deputy Director, Health Workforce Research Center, Fitzhugh Mullan Institute for Health Workforce Equity, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

43. Evans, Scott, PhD, MS, Director, The Biostatistics Center, Professor and Founding Chair, Department of Biostatistics and Bioinformatics, Milken Institute School of Public Health, The George Washington University

44. Feder, Judith, PhD, Professor, McCourt School of Public Policy, Georgetown University

45. Frankford, David M., JD, Professor of Law, Rutgers Law School

46. Freed, Salama, PhD, Assistant Professor, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

47. Friedman, Leonard H., PhD, MPH, FACHE, Professor and Director, MHA@GW Program, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

48. Gaba, Nancy D., MD, FACOG, Oscar I. and Mildred S. Dodek Professor and Chair, Department of Obstetrics and Gynecology, School of Medicine and Health Science, The George Washington University

49. Ganguli, Ishani, MD, MPH, Associate Professor of Medicine, Harvard Medical School

50. Ghosh, Mimi, PhD, MS, Associate Professor, Department of Epidemiology, Milken Institute School of Public Health, The George Washington University

51. Goldstein, Melissa M., JD, Professor, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

52. Grogan, Colleen M., PhD, Deborah R. and Edgar D. Jannotta Professor, Crown Family School of Social Work, Policy, and Practice, The University of Chicago

53. Hamilton, Bethany, JD, Director, National Center for Medical-Legal Partnership, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

54. Heinrich, Janet, DrPH, RN, FAAN, Research Professor, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

55. Hoffman, Allison K., JD, William Maul Measey Professor of Law and Health Sciences, Professor of Medical Ethics & Health Policy, University of Pennsylvania Carey Law School

56. Holtz, Timothy H., MD, MPH, FACP, FACPM, RDML (ret) US PHS, Redstone Chair & Director, Sumner M. Redstone Global Center for Prevention and Wellness, Professor, Department of Global Health & Department of Epidemiology, Milken Institute School of Public Health, The George Washington University

57. Horton, Katherine, RN, MPH, JD, Research Professor, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

58. Huberfeld, Nicole, Edward R. Utley Professor of Health Law, Co-Director, BU Program on Reproductive Justice, Chair, BU Health Law Program, Boston University School of Law and School of Public Health

59. Imershein, Sara, MD, MPH, FACOG, Clinical Professor, OB-GYN, School of Medicine and Health Sciences, The George Washington University

60. Jacobs, Feygele, DrPH, MS, MPH, Professor and Director, Geiger Gibson Program in Community Health, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

61. Jordan, Jeanne A., PhD, HCLD(ABB), MT(ASCP), Professor Emerita, Department of Epidemiology, Milken Institute School of Public Health, The George Washington University

62. Jost, Timothy Stoltzfus, JD, Professor Emeritus, Washington and Lee University School of Law

63. Katz, Ingrid, MD, MHS., Director, Yale Institute for Global Health

64. Ku, Leighton, PhD, MPH, Professor, Department of Health Policy and Management, Director, Center for Health Policy Research, Milken Institute School of Public Health, The George Washington University

65. Lantz, Paula, PhD, Associate Director, International Policy Center, James B. Hudak Professor of Health Policy, Director of BA Undergraduate Programs, Gerald R. Ford School of Public Policy, Professor of Health Management and Policy, University Professor of Diversity and Social Transformation, School of Public Health, University of Michigan

66. LaPuma, Peter, PhD, CIH, PE, Professor, Department of Environmental and Occupational Health, Milken Institute School of Public Health, The George Washington University

67. Law, Sylvia A., JD, Elizabeth K. Dollard Professor Emerita of Law, Medicine and Psychiatry, NYU Law School

68. Levi, Jeffrey, PhD, Professor Emeritus, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

69. Lewis-O'Connor, Annie, PhD, NP, MPH, Instructor in Medicine, Harvard Medical School, Founder and Director, C.A.R.E. Clinic, Brigham and Women's Hospital

70. Lillie-Blanton, Marsha, DrPH, Adjunct Professor, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

71. Magnus, Manya, PhD, MPH, Professor and Chair, Department of Epidemiology, Milken Institute School of Public Health, The George Washington University

72. Mariner, Wendy K., JD, LLM, MPH, Professor of Health Law, Ethics and Human Rights Emerita, Boston University School of Public Health

73. Markus, Anne R., PhD, MHS, JD, Professor and Chair, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

74. Mason, Diana J., RN, PhD, FAAN, Senior Policy Service Professor, Center for Health Policy and Media Engagement, School of Nursing, The George Washington University

75. McCarthy, Melissa L., ScD, MS, Professor of Health Policy and Emergency Medicine, Milken Institute School of Public Health, The George Washington University

76. McDonnell, Karen A., PhD, Associate Professor, Department of Prevention and Community Health, Milken Institute School of Public Health, The George Washington University

77. Michaels, David, PhD, MPH, Professor, Department of Environmental and Occupational Health, Milken Institute School of Public Health, The George Washington University

78. Michener, Jamila, PhD, Associate Professor of Government, Cornell University

79. Monroe, Anne K., MD, MSPH, Associate Professor, Department of Epidemiology, Milken Institute School of Public Health, The George Washington University

80. Murphy, Caitlin, MPP, Research Scientist, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

81. Musumeci, MaryBeth, JD, Associate Teaching Professor, Milken Institute School of Public Health, The George Washington University

82. Newcomer, Kathryn, PhD, MA, Professor of Public Policy and Public Administration, Trachtenberg School of Public Policy and Public Administration, The George Washington University

83. Oberlander, Jonathan, PhD, Professor, Department of Social Medicine, Professor, Department of Health Policy & Management, University of North Carolina at Chapel Hill

84. Parmet, Wendy E., JD, Matthews University Distinguished Professor of Law, Northeastern University

85. Perreira, Krista M., PhD, Professor, Department of Social Medicine, UNC School of Medicine

86. Perrin, James M., MD, Professor of Pediatrics Emeritus, Harvard Medical School, John C. Robinson Distinguished Chair in Pediatrics, MassGeneral for Children

87. Peterson, Mark A., PhD, Professor of Public Policy, Political Science, Health Policy and Management, and Law, UCLA Meyer and Renee Luskin School of Public Affairs

88. Pittman, Patricia, PhD, Professor of Health Policy and Management, Director of Health Workforce Research Center, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

89. Pollack, Harold, PhD, Helen Ross Distinguished Services Professor, Associated Faculty, Public Health Sciences, Crown Family School of Social Work, Policy, and Practice, Administration, University of Chicago

90. Poteat, Tonia, PhD, MPH, PA-C, Professor, Duke University School of Nursing, Research Professor of Global Health, Duke Global Health Institute

91. Price, Olga Acosta, Associate Professor and Vice Chair, Department of Prevention and Community Health, Milken Institute School of Public Health, The George Washington University

92. Reeves, Matthew, MD, MPH, FACOG, Chief Executive Officer, DuPont Clinic; Clinical Associate Professor of Obstetrics and Gynecology, George Washington University School of Medicine and Health Sciences; Adjunct Clinical Associate Professor of Obstetrics and Gynecology, Stanford University School of Medicine; Adjunct Associate Professor of Population, Family, and Reproductive Health, Johns Hopkins Bloomberg School of Public Health

93. Regenstein, Marsha, PhD, Professor, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

94. Rempe, Shannon, JD, MPH, Program Director, Advancing Public Health and Health Equity in the Courts, Northeastern University

95. Ricciardi, Richard, PhD, CRNP, FAAP, FAAN, Executive Director, Center for Health Policy and Media Engagement, Professor, School of Nursing, The George Washington University

96. Rich-Edwards, Janet, ScD, MPH, Associate Professor of Medicine, Harvard Medical School, Associate Professor in Epidemiology, Harvard T.H. Chan School of Public Health

97. Richards, Adam K., MD, PhD, MPH, DTM&H, Senior Associate, Department of International Health, Johns Hopkins Bloomberg School of Public Health

98. Riegelman, Richard, MD, MPH, PhD, Professor of Epidemiology and Founding Dean, Milken Institute School of Public Health, The George Washington University

99. Rittenberg, Eve, MD, Assistant Professor of Medicine, Harvard Medical School

100. Rocco, Philip, PhD, Associate Professor of Political Science, Marquette University

101. Rosenbaum, Sara, JD, Professor Emerita, Health Law and Policy, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

102. Sage, William, MD, JD, Professor of Law, Medicine, and (by courtesy) Government and Public Service, Assistant Vice President of the Health Science Center, Texas A&M University

103. Schneider, Andy, JD, Research Professor of the Practice, McCourt School of Public Policy, Georgetown University

104. Seiler, Naomi, JD, Professor, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

105. Silberman, Pam, JD, DrPH, Professor Emerita, Director, Executive Doctoral Program in Health Leadership, Department of Health Policy and Management, UNC Gillings School of Global Public Health

106. Silverman, Hannah, MPH, Senior Research Associate, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

107. Siminoff, Laura A., PhD, Laura H. Carnell Professor of Public Health, Department of Social and Behavioral Sciences, Temple University

108. Skinner, Daniel, PhD, Professor of Health Policy, Department of Social Medicine, Heritage College of Osteopathic Medicine, Ohio University

109. Slifkin, Becky, PhD, Professor Emerita, Department of Health Policy and Management, UNC Gillings School of Global Health

110. Sparer, Michael, JD, PhD, William Henry Welch Professor of Public Health, Department of Health Policy and Management, Columbia University Mailman School of Public Health

111. Strasser, Julia, DrPH, MPH, Assistant Research Professor, Department of Health Policy and Management, Director, Reproductive Health Workforce & Policy Research Center, Fitzhugh Mullan Institute for Health Workforce Equity, Milken Institute School of Public Health, The George Washington University

112. Suter, Sonia M., JD, MS, Professor of Law, The Kahan Family Research Professor of Law, Co-Director, Health Law & Policy Program, The George Washington University Law School

113. Swartz, Katherine, PhD, MS, Professor of Health Policy and Management, Emerita, Harvard T.H. Chan School of Public Health, Harvard University

114. Tavrow, Paula, Ph.D., Adjunct Professor, Department of Community Health Sciences, University of California Los Angeles Fielding School of Public Health

115. Vichare, Anushree, PhD, MBBS, MPH, Assistant Professor, Department of Health Policy and Management, Milken Institute School of Public Health, The George Washington University

116. Vyas, Amita N., PhD, MHS, Professor, Director, Maternal & Child Health Program, Department of Prevention and Community Health, Milken Institute School of Public Health, The George Washington University

117. Waldman, Ronald, MD, MPH, Professor Emeritus of Global Health, Milken Institute School of Public Health, The George Washington University

118. Warren, Keegan, JD, LLM, Executive Director, Institute for Healthcare Access, Texas A&M University Health Science Center

119. Wasserman, Alan G., MD, MACP, Eugene Meyer Emeritus Professor of Medicine, Department of Medicine, The George Washington School of Medicine and Health Sciences

120. Watson, Sidney D., JD, Jane and Bruce Robert Professor of Law and Scholar in Residence, Center for Health Law Studies, Saint Louis University School of Law

121. Westmoreland, Timothy M., JD, Professor from Practice, Emeritus, Georgetown University School of Law

122. Wise, Paul H., Richard E. Behrman Professor of Child Health and Society, Senior Fellow, Freeman Spogli Institute for International Studies, Core Faculty, Center on Democracy, Development and the Rule of Law, Affiliated faculty at the Center for International Security and Cooperation, Stanford University

123. Wisner, Katherine L., MD, MS, DLFAPA, Professor of Psychiatry and Pediatrics, Developing Brain Institute, Children's National Hospital, Professor of Obstetrics and Gynecology, George Washington University School of Medicine

124. Yang, Y. Tony, DSc, LLM, MPH, Endowed Professor of Health Policy, The George Washington University

125. Young, Heather A., PhD, MPH, Vice Chair/Professor/MPH Epidemiology Co-Director/PhD Epidemiology Director, Department of Epidemiology, Milken Institute School of Public Health, The George Washington University